

Neurology/OtoNeurology
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SENTA CLINIC

DIVISION OF NEUROLOGY/OTONEUROLOGY

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Name **TEST** Date:

DOB:

Neurology – Balance Disorders:
[✔] Consult [ ] Evaluation [ ] Progress Note [ ] Urgent Add-On

Consult requested by: CC: CC: CC:

PMD

HPI: Briefly, patient is a 0 year old [ ] Male [ ] Female with a history of: Allergies: [ ] NKDA
Chief Complaint: Date of onset:

Mr./Ms. **** is a *** y/o ***HD fe***male presenting to the clinic today for a consultation on ***** .
PMHx significant for **** Medications:

No other focal motor, sensory, or CN deficits reported at this time.

Ataxia
Balance Disorders
Balance Rehabilitation
Hearing Loss
Headache
Migraine
Migraine Vertigo
Otoneurology Presbyastasis
Tinnitus
Trigeminal Neuralgia
Vertigo
Vestibular Rehabilitation
Vestibular Testing
Vestibulopathies

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[ ] DIZZINESS AND LACK OF COORDINATION [ ] Patient Denies
Date of Onset:
Frequency:
Associated with:
[ ] Vertigo [ ] Chronic baseline disequilibrium
[ ] Disequilibrium [ ] Headache
[ ] Attacks [ ] Tinnitus
[ ] Hearing loss/fluctuation
[ ] Aural fullness [ ] AS [ ] AD [ ] AU
Symptom Course:
[ ] Paroxysmal [ ] Chronic
[ ] Stable [ ] Worsening [ ] Improving
Symptoms occur if:
[ ] Sitting [ ] Supine [ ] Standing [ ] Walking
Duration:
[ ] Last seconds [ ] Minutes [ ] Hours [ ] Days

Worse with:
[ ] Standing
[ ] Walking
[ ] Walking in dark
[ ] Trans. to standing from sitting or supine positions
[ ] Pitching head up/down
[ ] Yaw to left or right
[ ] Lying supine in bed
[ ] Stress/fatigue
[ ] Optokinetic hypersensitivity
[ ] Any high gain head accel. in nonspecific directions
Symptoms better if:
[ ] Sit
[ ] Lie supine
[ ] Hold head still in a neutral position
[ ] Resolves on own
[ ] Take Medications:
[ ] Other relieving factors:
[ ] No clear relieving or exacerbating factors

[ ] HEARING LOSS [ ] Patient Denies
Date of onset:
[ ] Right [ ] Left [ ] Bilateral
Affects ADL's
[ ] Mild [ ] Moderate [ ] Severe
Symptom Course:
[ ] Paroxysmal [ ] Chronic [ ] Stable [ ] Worsening
[ ] Improving [ ] Attacks
Attacks Associated with:
[ ] Vertigo [ ] Disequilibrium [ ] Headache [ ] Tinnitus
[ ] Hearing Fluctuation [ ] Aural fullness
[ ] None of the above

Symptoms better if:
[ ] Wear hearing aids
[ ] Other
[ ] No clear relieving or exacerbating factors

Worse if:
[ ] Loud background noise
[ ] Other

Duration:
[ ] Last seconds [ ] Minutes [ ] Hours [ ] Days

[ ] TINNITUS [ ] Patient Denies
Date of onset:
[ ] Pulsatile [ ] Non-Pulsatile
[ ] Right [ ] Left [ ] Bilateral
Affects ADL's:
[ ] Mild [ ] Moderate [ ] Severe
Symptom Course:
[ ] Paroxysmal [ ] Chronic
[ ] Stable [ ] Worsening [ ] Improving [ ] Attacks
Frequency [ ] High [ ] Med [ ] Low
Attacks Associated with:
[ ] Vertigo [ ] Disequilibrium
[ ] Headache [ ] Hearing loss [ ] Fluctuation [ ] Aural fullness
[ ] None of the above

Symptoms better if:
[ ] Wear hearing aids
[ ] Other
[ ] No clear relieving factors

Worse if:
[ ] Loud background noise
[ ] Other
[ ] No clear exacerbating factors

Duration:
[ ] Last seconds [ ] Minutes [ ] Hours [ ] Days

[ ] GAIT ATAXIA [ ] Patient Denies
Date of onset:
Symptom Course:
[ ] Paroxysmal [ ] Chronic
[ ] Stable [ ] Worsening [ ] Improving [ ] Attacks
Associated with:
[ ] Orthostasis [ ] Veer to left [ ] Veer to right [ ] Bradykinesia
[ ] Headache [ ] Mental status decline
[ ] Weakness of legs [ ] denies
[ ] Sensory loss in legs [ ] denies
[ ] Bowel incontinence [ ] denies
[ ] Bladder incontinence [ ] denies

Symptoms better if:
[ ] Use Cane Duration:
[ ] Use Walker Duration:
[ ] Wheelchair Duration:
[ ] Sit or Rest
[ ] No clear relieving factors

Worse if:
[ ] Walk
[ ] Turn corners quickly
[ ] Other
[ ] No clear exacerbating factors

Falls: [ ] Yes [ ] No

[ ] HEADACHE [ ] Patient Denies
Date of onset:
[ ] Right [ ] Left-sided [ ] Global [ ] Frontal [ ] Occipital
Affects ADL's
[ ] Mild [ ] Moderate [ ] Severe Scale /10
Symptom Course:
[ ] Paroxysmal [ ] Chronic Frequency: /month /week
[ ] Stable [ ] Worsening [ ] Improving [ ] Attacks
[ ] Sharp [ ] Dull [ ] Throbbing [ ] Constant [ ] Pressure
Associated with:
[ ] Vertigo [ ] Disequilibrium [ ] Visual Scotomas [ ] Aura
[ ] Hearing loss [ ] Fluctuation [ ] Aural fullness [ ] Tinnitus
[ ] Nausea [ ] Vomiting [ ] Photophobia [ ] Phonophobia
[ ] None of the above

Symptoms worse if:
[ ] Dietary
[ ] Lights
[ ] No clear relieving factors

[ ] Loud background noise
[ ] Stress / fatigue
[ ] No clear exacerbating factors

Duration:
[ ] Last seconds [ ] Minutes [ ] Hours [ ] Days

Symptoms Better if:
[ ] Medications:
[ ] Hydration therapy
[ ] Other:

LABS:

PAST MEDICAL HISTORY / PAST SURGICAL HISTORY:
[✔] Also see patient questionnaire
[ ] Hypertension
[ ] Dyslipidemia
[ ] Diabetes
[ ] Stroke
[ ] Other
[ ] Migraine headaches
[ ] Coronary Artery disease
[ ] Cardiac Arrhythmias
[ ] Cervical disease

[ ] No changes in follow-up
[ ] Hx of head trauma w/ LOC [✔] denies
[ ] History of severe ear infections [✔] denies
[ ] Hx of ototoxic drug exposure [✔] denies
[ ] Last menstrual period:

FAMILY HISTORY:
[✔] Also see patient questionnaire
[ ] No changes in follow-up
Mother: [ ] Alive [ ] Deceased Age_____ History:
Father: [ ] Alive [ ] Deceased Age_____ History:

OTHER:

SOCIAL HISTORY:
[✔] Also see patient questionnaire. [ ] No changes in follow-up
[ ] Denies Tobacco/ETOH/Drugs
[ ] Tobacco: _____ pack/day [ ]Remote
[ ] Etoh [ ] Alcohol Abuse history [ ] Remote
[ ] Drugs [ ] Remote

REVIEW OF SYSTEMS:
[✔] See patient questionnaire dated today
[✔] Full 10 System ROS performed and (-) except for as mentioned below

 Patient Name: **TEST** DOB: Date:

Otoneurologic Exam: On Exam today, I find (* Note: check or vertical line through normal means item was assessed, slash means not assessed): X means abnormal

Constitutional	BP:	HR:	RR:	Weight:	Height:	BMI:	Temperature:	° Pain: /10

		Normal	Pertinent Positive Findings
	Appearance is WD / WN / NAD	☑	
Head	Normocephalic/ Atraumatic	☑	
Eyes	Fundi visualized no papilledema or hemorrhage	☑	
	Visual fields intact	☑	
	PERRLA	☑	
ENT	Oropharynx	☑	
	Tympanic membranes & ear canals w/ microscopy	☑	
Cardiovascular	Regular rate & rhythm	☑	
	No murmurs or carotid bruits	☑	
Lungs	Clear to auscultation bilaterally	☑	
Extremities	No clubbing / cyanosis / edema	☑	
Neurologic	Oriented to person / place / time	☑	
	Intact recent & remote memory	☑	
	Appropriate mood & affect	☑	
	Normal attention span/ concentrate	☑	
	Normal language functions		
	Normal fund of knowledge	☑	
	No neglect or inattention	☑	
Cranial Nerves	Cranial nerve II	☑	
	Cranial nerve III, IV & VI	☑	
	Cranial nerve V	☑	
	Cranial nerve VII	☑	
	Cranial nerve VIII Otoneurologic exam [ ] Keratinous debris [ ] cerumen occluding tympanic membrane	☑	Weber: Midline Rinne A>B: AU Rinne B>A:
	Cranial nerve IX & X	☑	
	Cranial nerve XI	☑	
	Cranial nerve XII	☑	
Motor	5/5 strength x 4 extremities	☑	
	Normal tone throughout	☑	
Sensory	Intact touch / position / vibration	☑	
	Intact pin / 10 g monofilament / temperature	☑	
Reflexes	DTRs symmetric in all 4 extremities	☑	(R) ___Bi ___Tri ___BR ___Pat ___Ach (L) ___Bi ___Tri ___BR ___Pat ___Ach
	Toes downgoing bilaterally	☑	
Coordination	No ataxia on FTN and HTS	☑	
	Normal rapid finger movements	☑	
Gait	Normal based gait	☑	

Video Infrared Goggle Study:
Performed directly on the patient by: Date:
Performed to mask visual suppression secondary to balance complaints where a diagnosis of an atypical Nystagmus would allow proper diagnosis, a focal workup, and management of the patient.

Dix-Hallpike Testing:
Head Right: [ ] Normal [ ] Torsional & vertical Nystagmus [ ] Fatiguing [ ] Latency [ ] Atypical
Head Left: [ ] Normal [ ] Torsional & vertical Nystagmus [ ] Fatiguing [ ] Latency [ ] Atypical

Supine Positional testing:
Head Center: [ ] Normal [ ] Left Beat [ ] Right Beat [ ] Down Beat [ ] Upbeat [ ] Atypical
Head Right: [ ] Normal [ ] Geotropic [ ] Ageotropic [ ] Down Beat [ ] Upbeat [ ] Atypical
Head Left: [ ] Normal [ ] Geotropic [ ] Ageotropic [ ] Down Beat [ ] Upbeat [ ] Atypical

Gaze Evoked Study:
Gaze Right: [ ] Normal [ ] Left Evoked [ ] Right Evoked
Gaze Left: [ ] Normal [ ] Left Evoked [ ] Right Evoked

Comments/ treatment:

[ ] See Epley Chair/CRM Chair Report

Study Impression:
[ ] Normal Study
[ ] Right Posterior Canal BPPV
[ ] Left Posterior Canal BPPV
[ ] Atypical Nystagmus
[ ] Right Cupulolithiasis
[ ] Left Cupulolithiasis
[ ] Right Sustained Nystagmus
[ ] Left Sustained Nystagmus
[ ] Left Lateral Canal BPPV
[ ] Right Lateral Canal BPPV
[ ] Left Anterior Canal BPPV
[ ] Right Anterior Canal BPPV

Imaging / Studies	Date	Result

IMPRESSION/PLAN:

Total encounter time of 60 minutes, of which included direct patient counseling; previsit review of records to include imaging studies, outside records and/or laboratory data; ordering and coordinating treatment care/medication management; documentation.

Additionally, I spent up to

[ ] 15 minutes of separate time IR/oculography and/or Epley Chair testing
[ ] 15 minutes of separate time for Injection Procedures

PLAN: [ ] Balance log [ ] HA log [ ] Fall precautions [ ] Continue home vis/vest exercises

[ ] Medications [ ] Patient advised to use two forms of birth control while on this/these medications.
[ ] We discussed the indications / contraindications / adverse affects of
[ ] Keep head greater than earth horizontal [ ] Sleep head 45 degrees [✔] Return to clinic ASAP vertigo / worsening symptoms
[ ] Patient instructed not to drive when experiencing episodes of vertigo or disequilibrium

Next Appointment: [ ] Days [ ] Weeks [ ] Months

Referrals:

Physician:
Specialty:
Diagnosis:

Physician:
Specialty:
Diagnosis:

Imaging/Orders:

[ ] PT

Provider

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[ ]